Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
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INFORMED CONSENT FORM

Submental Liposuction

Removal of fat under the chin (double-chin liposuction)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Planned technique: ☐ Conventional (tumescent) ☐ Energy-assisted (laser/RF) ☐ To be decided by the surgeon

Anaesthesia: ☐ Local anaesthesia ☐ Local anaesthesia + sedation ☐ General anaesthesia _____

Procedures performed at the same time: ☐ None ☐ With a separate consent form: _____

Patient's concerns and expectations: _____

1 Your condition and the aim of the procedure

Fat collecting under the chin (a “double chin”) can blur the angle between the chin and neck, making the jawline look less defined, tired or heavier than it really is. Fat in this area is often due to a **genetic tendency** and may not disappear completely even with weight loss or exercise. The loss of skin elasticity with age also makes it more noticeable.

Submental liposuction removes the **fat beneath the skin** by suction through fine cannulas. The aim is to define the jawline and achieve a **natural, balanced chin–neck angle**.

The most important factor determining the result is **skin quality and elasticity**. Elastic skin adapts to the new contour after fat removal; skin with little elasticity may show more laxity. Deep fat beneath the neck muscle (platysma), large salivary glands, prominent neck bands and severe skin laxity cannot be corrected with liposuction; in these cases other methods, such as a deep neck lift, may be needed.

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2 How is the procedure performed?

Before surgery your chin and neck are examined, skin elasticity is assessed, photographs are taken and the area to be treated is marked with you sitting upright.

1. **Numbing and fluid infiltration:** A special fluid (tumescent solution) containing local anaesthetic and a medicine to reduce bleeding is injected into the area. This fluid prevents pain and also reduces bleeding and bruising.
2. **Small incisions:** Small entry points of about **3–5 mm** are made in the natural crease under the chin and, if needed, behind the earlobes.
3. **Fat removal:** The fat is carefully and evenly removed by suction through fine cannulas (metal tubes). A thin layer of fat is left under the skin to keep the surface smooth.
4. **Energy-assisted treatment (if planned):** Laser or radiofrequency energy is used to soften the fat and help the skin tighten to some degree. This may contribute to skin tightening, but it does not correct significant laxity.
5. **Closure and garment:** The entry points are closed with a single stitch or left to close on their own. At the end of the operation an **elastic compression garment (chin strap)** is placed around the chin and neck.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
Local anaesthesia, local + sedation, or general anaesthesia	About 45–90 minutes (when done on its own)	Usually same-day discharge

3 Expected benefits and likelihood of success

- Reduction of excess fat under the chin
- A more defined jawline and chin–neck angle; a more balanced facial profile
- A **long-lasting** result as long as your weight remains stable, because the fat cells removed do not grow back (natural ageing changes in the skin and tissues will continue)

Most patients with good skin elasticity are satisfied with the result. The result becomes apparent within **3–6 months** as swelling settles. About **3–5 in 100** patients may need a small touch-up procedure because of irregularity, residual fat or asymmetry. Significant weight gain and ageing can affect the result over time.

4 Other treatment options

- **Not having the procedure:** As this is a cosmetic procedure, you always have the option not to have it.
- **Weight loss and lifestyle changes:** Reduce overall body fat but do not always remove stubborn fat under the chin.
- **Fat-dissolving injections (e.g. deoxycholic acid):** Require several sessions, can cause marked swelling and pain, and have a more limited effect.
- **Fat freezing (cryolipolysis) and device treatments:** Non-surgical, but require several sessions and have a more limited effect.
- **Deep neck lift / neck lift surgery:** A more suitable option if there is significant skin laxity, neck bands or deep fat beneath the muscle.

5 What happens if the procedure is not performed?

If the procedure is not performed, the appearance under your chin will remain as it is and may increase with age or weight gain. This poses no danger to your health.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

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6 Preparing for the procedure

- Blood tests are carried out before surgery; if sedation or general anaesthesia is planned, you will be assessed by the anaesthetist and will need to fast beforehand.
- Your weight should ideally be stable before the procedure; if you plan significant weight change in the near future, discuss this with your doctor.
- Do not apply cream or make-up to your face and neck on the day of surgery; wear a comfortable top that opens at the front rather than a high-necked one.
- After sedation or general anaesthesia you must not drive; please go home with a companion.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 48 hours	There will be swelling, tightness and mild pain under the chin, and some pinkish fluid may leak from the entry points; this is normal. Bruising, **if it occurs**, may spread down the neck and settles within 1–2 weeks. Sleep with your head raised and wear the garment continuously.
Week 1	The garment is usually worn **day and night for the first week** and then **at night for a further 2–3 weeks**. Any stitches are removed on day 5–7. Most patients return to desk work and social life within 3–7 days.
Weeks 2–6	Swelling largely subsides. Firmness, small lumps and numbness may be felt in the area; these soften with time. Strenuous exercise can usually be resumed after **2–3 weeks**. If recommended, gently massage the area.
Months 3–6	Remaining swelling and firmness resolve, the skin adapts to the new contour and the result becomes clear.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Swelling, tightness and mild pain under the chin and in the neck; bruising, **if it occurs**, settles within 1–2 weeks
- Pinkish fluid leaking from the entry points for the first 24–48 hours
- Temporary numbness and reduced sensation in the area
- Mild firmness and small palpable lumps lasting several weeks

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Common conditions (most are temporary or resolve with treatment)

- **Prolonged firmness and lumps (fibrosis)** — Firmness and lumpiness under the skin are common during healing; most soften within a few months with massage and time.
- **Prolonged numbness** — Reduced sensation of the skin under the chin may last a few weeks to a few months; very rarely it is permanent.
- **Mild asymmetry** — Because the two halves of the face are never perfectly symmetrical, small differences may remain.

Uncommon conditions

- **Contour irregularity and dimpling** — About **2–5 in 100** patients may have rippling, dents or residual fat deposits. If too much fat is removed, a hollow or "skeletonised" appearance under the chin and more visible neck bands may result. Correction may require fat injection, further liposuction or another procedure.
- **Skin laxity or insufficient tightening** — Particularly in people with reduced skin elasticity, the skin may not tighten enough after fat removal and laxity may look more noticeable. In this case a neck lift may be needed in the future.
- **Asymmetry of the smile (effect on the nerve branch to the lower lip)** — A fine nerve branch running along the jawline that moves the lower lip may be affected by swelling or pressure, causing asymmetry of the lower lip when smiling. This occurs in about **1 in 100** patients and almost always recovers on its own **within a few weeks to a few months**; permanent weakness is very rare.
- **Collection of blood or fluid (haematoma, seroma)** — Occurs in **fewer than 1 in 100** patients. Small collections resolve on their own; larger ones may need drainage with a needle or a small procedure.
- **Infection** — Occurs in **fewer than 1 in 100** patients; it usually improves with antibiotics, and rarely an abscess may need to be drained.
- **Scarring or discolouration at the entry points** — The small entry scars usually become inconspicuous; rarely darkening, thickening or a small dent may remain.

Rare or very rare, but potentially serious conditions

- **Skin burn (with energy-assisted techniques)** — When laser or radiofrequency is used, despite safety measures, a skin burn, blistering and permanent scarring may rarely occur; wound care and scar treatment may be needed.
- **Skin damage or tissue loss** — Very rarely, particularly in smokers, impaired blood supply to the skin may lead to a wound and scarring.
- **Permanent nerve damage** — Permanent weakness of lower lip movement is extremely rare; if it occurs, further treatment may be needed.
- **Reaction to the local anaesthetic** — An allergy to the local anaesthetic may occur, or a high blood level of it may cause dizziness, ringing in the ears, a metallic taste in the mouth and, very rarely, seizures or a heart rhythm disturbance. Doses are kept within safe limits.
- **Fat embolism and injury to deeper structures or the airway** — Fat droplets entering the circulation, or the cannula damaging deeper structures, are **extremely rare** in small-volume submental liposuction; if they occurred, emergency hospital and intensive care treatment could be required.
- **Psychological dissatisfaction** — Despite a technically successful result, a person may find it hard to adjust to their new appearance. Discussing expectations openly before surgery reduces this risk.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.

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- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

9 When to contact us immediately

- **Rapidly increasing, hard, tense swelling** of the neck, especially on one side
- **Difficulty breathing or swallowing**, or a change in your voice
- **Fever above 38.5 °C**, increasing pain, redness, warmth or foul-smelling discharge
- Skin turning white, dark purple or black; fluid-filled blisters
- Chest pain, shortness of breath or confusion

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- Wear the garment regularly for the period advised; make sure it is neither too tight nor too loose.
- Sleep on your back with your head raised for the first 1–2 weeks; avoid sudden or excessive stretching of the neck.
- Avoid strenuous exercise, saunas, Turkish baths and long hot showers for the first 2–3 weeks.
- Protect the area from the sun in the first months; this reduces the risk of scarring and discolouration.
- Try to keep your weight stable to maintain the result.
- If you have travelled from abroad, flying is usually safe from day 2–3 onwards.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

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11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

- ☐ I agree that my images are used **only for medical records and treatment follow-up**.
- ☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).
- ☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.
- ☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write “none”.

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.

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- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that the result depends on my skin elasticity, and that liposuction does not correct skin laxity, neck bands or deep fat beneath the muscle.
- ☐ I understand that wearing the garment and following the advice given will directly affect the result.
- ☐ I understand that this procedure is **cosmetic**, that it is not medically necessary and that I always have the option not to have it. I have discussed my expectations with my doctor, and the realistic result that can be achieved has been explained to me.
- ☐ I understand that perfect symmetry or an exact copy of the appearance in a photograph cannot be guaranteed, that the final result depends on how my tissues heal, and that in some cases a correction procedure may be needed.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

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FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.